CareOps General Hospital: Hospital Profile, Departments and Capacity

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1. Purpose and scope

This document describes CareOps General Hospital, a fictional 400-bed multispecialty teaching hospital that is used as the reference hospital for the CareOps AI operations intelligence prototype. All numbers, names and policies in the CareOps General Hospital documents are synthetic hospital policy created for a decision-support prototype, except where a statement explicitly names an external public standard such as the World Health Organization or the Biomedical Waste Management Rules 2016 of India. The hospital documents cover hospital operations only: patient flow, beds, staffing, equipment, support services and safety procedures. They do not contain medical diagnosis or treatment advice, and the AI assistant must not use them for that purpose.

2. Inpatient bed capacity by department

CareOps General Hospital has 400 licensed inpatient beds. The Emergency Department has 40 beds, which include resuscitation bays, treatment cubicles and observation beds. The Intensive Care Unit has 40 beds, divided into 20 Medical ICU beds, 12 Surgical ICU beds and 8 Cardiac ICU beds. The Neonatal ICU has 10 beds. General Medicine has 80 beds. General Surgery has 60 beds. Orthopedics has 40 beds. The Cardiology ward has 30 beds. Pediatrics has 40 beds. Obstetrics and Gynecology has 40 beds. The Isolation Unit has 20 beds, of which 4 are negative pressure airborne infection isolation rooms. These department bed counts add up to 400 beds.

3. Surge capacity

In addition to the 400 licensed beds, the hospital can open up to 40 surge beds during a declared capacity emergency. Twenty surge beds are located in the Day Care and Recovery area and twenty are located in designated ward corridors with portable oxygen and monitoring support. Surge beds may only be opened with approval from the Duty Manager at Level 2 and from the Medical Superintendent at Level 3 of the Overcrowding and Surge Capacity Plan.

4. Other clinical and support facilities

The hospital has 8 operating theatres. Seven are used for scheduled surgery and one is a dedicated emergency operating theatre that is kept free for urgent cases. The Outpatient Department has 24 consultation rooms. The Radiology department provides X-ray, ultrasound, 2 CT scanners and 1 MRI scanner. The clinical laboratory and the blood bank work 24 hours a day, every day. The central pharmacy and the Emergency Department satellite pharmacy work 24 hours a day. The hospital also has a Central Sterile Supply Department, a laundry, a kitchen, and a Biomedical Engineering workshop.

5. Operating hours and shifts

The Emergency Department, inpatient wards, ICUs, laboratory, blood bank and pharmacy operate 24 hours a day, 7 days a week. The Outpatient Department operates from 08:00 to

20:00 from Monday to Saturday and is closed on Sundays except for the Emergency Department. Nursing and support staff work three shifts: the Morning shift from 07:00 to 15:00, the Evening shift from 15:00 to 23:00 and the Night shift from 23:00 to 07:00. Consultant doctors work day duty and are on call at night.

6. Workforce summary

The hospital employs about 180 doctors including consultants and resident doctors, 520 nurses, 150 allied health professionals such as technicians, pharmacists and physiotherapists, and 200 support staff such as housekeeping, security and porters. A float pool of 30 nurses can be moved between departments to cover shortages.

7. Typical daily operational volumes

In normal conditions the Emergency Department receives 500 to 700 patient visits per day, which is usually 20 to 40 arrivals per hour averaged across the day, with the highest arrivals between 10:00 and 14:00 and between 18:00 and 22:00. The Outpatient Department sees 1,200 to 1,800 visits per day. The hospital admits about 70 to 90 patients per day. The average length of stay for inpatients is 4.2 days. The target hospital-wide bed occupancy is between 75 percent and 85 percent. Typical ED average waiting time from arrival to first doctor contact is 20 to 30 minutes.

8. Departments and their operational role

The Emergency Department treats unplanned and urgent arrivals and is the main source of unplanned admissions. The ICU cares for patients who need continuous monitoring or organ support and is the most resource-intensive unit per bed. General Medicine and General Surgery are the largest inpatient departments and receive most admissions from the Emergency Department. Orthopedics and Cardiology have a mix of scheduled and emergency admissions. Pediatrics and Obstetrics and Gynecology have separate emergency entry routes so that children and pregnant patients do not wait in the general triage queue. The Isolation Unit holds patients who need transmission-based precautions. The operating theatres and the Central Sterile Supply Department depend on each other, because theatre capacity is limited by the availability of sterile instrument sets.

9. Roles used in escalation

The Charge Nurse leads nursing in one ward or unit during a shift. The Bed Manager allocates beds across the hospital and runs the bed huddles. The Duty Manager is the senior administrative officer on duty and has authority over operational decisions during the shift. The Medical Superintendent is the head of clinical operations and declares the highest capacity level. The Hospital Administrator is the head of the hospital and is informed of Level 3 events. The Biomedical Engineer on duty manages equipment faults. The Infection Control Officer manages isolation and outbreak questions.